

Managed Hospital Asset Management Service

Asset Software + On-Site Asset Management Support for Small & Mid-Sized Hospitals

The Need

Hospitals invest significantly in medical equipment, furniture, IT hardware, engineering equipment and other movable assets. However, many small and mid-sized hospitals do not have a dedicated asset management team.

While asset management software can provide the technology platform, software by itself does not ensure that:

- Every asset is identified and entered correctly.
- Asset details and supporting documents are updated.
- A responsible custodian is assigned.
- AMC/CMC and warranty dates are monitored.
- Preventive maintenance is completed on time.
- Asset movement is recorded.
- Depreciation and current asset value are updated.
- Physical assets are periodically reconciled with the asset register.
- Missing, damaged, obsolete or underutilised assets are identified.
- Management receives reliable information on its asset base.

The proposed service addresses this gap by combining **Asset Management Software with professional on-site asset management support.**

Proposed Solution

Macula Healthcare can offer hospitals an end-to-end **Managed Asset Management Service**, where the hospital gets both the technology platform and a professional service team responsible for maintaining the asset management system.

The objective is simple:

“We don't just provide the asset software. We take responsibility for keeping the hospital's asset register accurate, current and auditable.”

Scope of Service

1. Initial Asset Onboarding

The engagement begins with creation of the hospital's complete digital asset register.

The team will:

- Create the hospital account and asset hierarchy.
- Define hospital → building → floor → department → location structure.
- Physically identify and catalogue assets.
- Generate/attach unique Asset IDs.
- Upload available invoices, purchase details and supporting documents.
- Record make, model, serial number, supplier and purchase date.
- Capture warranty/AMC/CMC details.
- Record purchase value and depreciation parameters.
- Identify present location and operational status.
- Assign department and individual custodian wherever applicable.

The outcome will be a **digitally verified Master Asset Register**.

2. Asset Lifecycle Management

Once assets are onboarded, the system will track the complete asset lifecycle:

Procurement → Installation → Allocation → Utilisation → Maintenance → Transfer → Depreciation → Replacement/Disposal

Important triggers can include:

- Warranty expiry
- AMC/CMC renewal
- Preventive maintenance due dates
- Calibration due dates
- Insurance renewal, where applicable
- Statutory inspection requirements
- Asset transfer
- Asset age
- End-of-life/replacement alerts

3. Custodian-Based Accountability

Each identified asset can be assigned to a department or designated custodian.

The system will therefore answer three basic management questions:

What asset do we own?

Where is it located?

Who is responsible for it?

Movement of important assets between departments can also be recorded, creating greater accountability.

4. Asset Valuation & Depreciation

Purchase value and applicable depreciation methodology can be entered into the system to provide management with:

- Original acquisition value
- Accumulated depreciation
- Current book value
- Asset age
- Remaining useful life
- Replacement requirement

This creates a useful bridge between **hospital operations, biomedical engineering and finance/accounts**.

5. Monthly Self-Audit

A structured monthly asset verification can be carried out by the hospital team with support from the managed service.

The monthly review can identify:

- Assets not found
- Assets shifted from assigned locations
- Custodian changes
- Non-functional equipment
- Equipment awaiting repair
- AMC/CMC issues
- Preventive maintenance overdue
- Calibration overdue
- Newly purchased assets not yet registered
- Assets proposed for condemnation/disposal

Exceptions will be highlighted rather than requiring management to review the entire asset register.

6. Quarterly Major Asset Audit

A more comprehensive physical asset audit can be conducted every quarter.

The quarterly audit will reconcile:

Physical Asset ↔ Asset Register ↔ Department ↔ Custodian ↔ Maintenance Status ↔ Financial Record

A quarterly Asset Management Report can then be presented to hospital management.

Management Dashboard

The hospital management should be able to view key information such as:

- Total number of assets
- Total acquisition value
- Present depreciated/book value
- Department-wise asset value
- Medical vs non-medical assets
- Assets under warranty
- Assets under AMC/CMC
- AMC/CMC due in next 30/60/90 days
- Preventive maintenance overdue
- Calibration overdue
- Non-functional assets
- Assets due for replacement
- Missing/unverified assets
- Asset utilisation/idle assets, wherever measurable

This converts the asset register from an accounting requirement into a **management decision-support system**.

Commercial Model

The service can have two components.

A. One-Time Asset Onboarding Fee

Charged for:

Physical asset verification + tagging + data collection + document uploading + asset register creation + custodian mapping + initial reconciliation.

Pricing can potentially be based on the **number of assets** or size of the hospital.

B. Annual Managed Asset Service

A recurring subscription combining:

Software Licence + Asset Management Service + Periodic On-Site Support + Audit + Management Reporting

A tiered model could be developed for:

Up to 50 beds | 51–100 beds | 101–200 beds | 200+ beds

Additional services such as complete physical audits, asset valuation exercises, biomedical equipment audits or large-scale asset tagging can be separately charged.

Value Proposition for Smaller Hospitals

The proposition should not be sold primarily as another software product.

The stronger positioning is:

“Your outsourced Asset Management Department.”

Instead of employing a dedicated asset manager, biomedical resource and administrative support solely for asset tracking, a smaller hospital gets a structured technology-enabled service at a predictable cost.

The hospital gains:

Better Asset Control | Reduced AMC Leakage | Preventive Maintenance Compliance | Custodian Accountability | Accurate Asset Register | Financial Visibility | Audit Readiness | Better Replacement Planning

Strategic Opportunity

This can eventually become more than an asset-tracking service.

Once Macula manages the asset database, the platform can support hospitals with:

Annual equipment budgeting → Replacement planning → AMC/CMC negotiations → Vendor performance monitoring → Equipment procurement → Disposal planning → CAPEX planning

This creates a recurring relationship with the hospital throughout the lifecycle of its medical and non-medical assets.

Suggested Service Name

Macula AssetCare™

Managed Hospital Asset Lifecycle Services

Alternative positioning:

“We manage your hospital assets, while you manage patient care.”